

Purpose of the evaluation

Social Security Disability Insurance (SSDI) is a federal program intended to help people who cannot work due to a severe medical condition with function-related financial support. The term "disability insurance", however, does not mean it is restricted to individuals with visible or permanent disabilities. SSDI is open to any individual who has a medical condition that meets Social Security's two-part disability definition. The program covers many different physical or mental health conditions that severely limit a person's ability to work and are expected to last for at least one year or end in death.

One of the most critical barriers for SSDI recipients to obtain Medicare coverage is the 24-month waiting period. This policy was created in an effort to control government expenditure and maintain separation of employer-sponsored insurance and federal government insurance.

However, many SSDI recipients do not have any other access to any form of healthcare coverage during this period. This gap often leads to voids in access to medical care leaving people in an untenable and precarious situation.

For many recipients, this waiting period creates severe stress on their financial situation, as well as their health that not only affects them but also their family. Some of those receiving SSDI benefits are likely to lose their homes, while many are struggling to manage chronic or deteriorating conditions. Because they cannot reliably access doctors, prescriptions, or treatments, they will suffer from an avoidable deterioration due to lack of timely treatment. As time progresses, the likelihood of worsening conditions increases, preventing individuals from improving their health and leading to more severe, long-term health issues that are more expensive to treat.

Studies show that SSDI beneficiaries experience a number of obstacles associated with the 24-month waiting for Medicare benefits. In their article *Costs and Benefits of Eliminating the Medicare Waiting Period for SSDI Benefits*, authors Gina Livermore, David Stapleton, and Henry Claypool (2009) discuss the severe financial consequences experienced by many SSDI beneficiaries. For many people, the only way to have health care insurance during this waiting period is through COBRA. Unfortunately, the expensive premiums of COBRA insurance create a financial burden for many SSDI beneficiaries that causes them to avoid essential development of health care through structured reimbursement. In this situation, many recipients postpone or abandon (due to imposed costs) visits to any provider of healthcare services further compounding their health.

In fact, 24% of SSDI recipients report they were unable to receive needed medical care during the waiting period, which led to poorer health and emotional turmoil (Livermore, Stapleton, & Claypool, 2009). The fact that individuals cannot access medical care in a timely manner so that they can manage or treat their medical conditions means that these conditions often become more severe and create greater health loss and are more costlier from a healthcare standpoint. As individuals' medical conditions become more severe, and they eventually seek care, the cost of care is usually higher and more intensive, creating greater health strain on the SSDI recipients and the healthcare system.

The lack of access to healthcare during this waiting period impacts more than just individuals but also has broader implications on society. The financial burden for SSDI recipients can contribute to additional stress related to having to make the choice of affording basic healthcare services versus affording basic living needs. In many cases, this stress may contribute to other personal struggles such as housing loss or lack of financial security which only increases and prolongs the

negative impact on health. Additionally, the longer individuals are without adequate healthcare, there is a greater chance their medical issue and subsequent treatment become increasingly more expensive.

Livermore, Stapleton, and Claypool (2009) argue that removing the waiting period would provide immediate access to healthcare, and thus improve health outcomes for SSDI recipients. Specifically, it would allow people to address their health issues in a timely manner and avoid the development of co-morbidities that are more complex and expensive to treat and affect overall health and quality of life. Immediate healthcare coverage would also help lessen the burden and expenditure of state Medicaid programs. Timely access to Medicare would minimize SSDI recipients from relying on Medicaid for interim care, and reduce difficulty with access to healthcare and delist the reliance on public healthcare systems therefore decreasing overall costs. Consequently, making timely healthcare available to SSDI recipients would benefit them individually, and the healthcare system as a whole.

Although the elimination of the Medicare waiting period certainly has its merits, the current model still requires SSDI recipients to experience a 24-month wait before they have coverage. The SSA has provided several options for programs in recognition of the difficulty of the gap, but these programs often fail to meet beneficiaries' healthcare needs.

The Social Security Administration (SSA) proposed a few healthcare programs that could help SSDI beneficiaries while they wait for the 24-month waiting period before they qualify for Medicare. These programs are designed to provide healthcare options, but most of them are very restrictive in which many beneficiaries may have no access to care during this critical time. One of the primary options offered by SSDI is Medicaid, a program that provides health insurance coverage for low-income individuals, including persons with disabilities, which is

funded both by the state and the federal government. However, Medicaid eligibility is not consistent across states. In states that expanded Medicaid as part of the Affordable Care Act (ACA), SSDI beneficiaries may be able to get health coverage based on income and medical necessity, but in states that did not expand Medicaid, SSDI beneficiaries may have no access to this program at all, or may be excluded from Medicaid, as Medicaid is not required to cover all disabilities, they may simply not qualify at all. The eligibility process can be complicated for beneficiaries even in expansion states, and there may still be barriers to accessing benefits for some SSDI recipients, especially those with chronic disabling conditions. Medicaid benefits differ by state, so there are going to be limited or non-existent services among states for particular healthcare services like transportation and home health, which are deemed necessary healthcare.

Another program recommended by SSDI is the Children's Health Insurance Program (CHIP). Originally developed as a program to cover children from low-income households, CHIP can also, in some states, cover pregnant women, and in rare cases, low-income adults. However, it is of little value to SSDI recipients because it primarily addresses children and, if they do not fall into these categories, there are limited benefits for them in CHIP. Others who are eligible for CHIP can receive coverage that will not address their full health care needs, particularly those with complex or long-term disabilities. Thus, CHIP represents a limited resource for many SSDI beneficiaries.

Another option for SSDI recipients who cannot qualify for Medicaid or CHIP is the ACA Marketplace. The ACA Marketplace allows individuals to purchase health insurance plans. Many people qualify for subsidies based on their income. For some people this can be a reasonable option, but typically the premiums and out of pocket expenses are very high and

SSDI recipients are usually already having financial difficulties, which is why they are eligible for SSDI, and may find it especially difficult to afford these plans. While it is true that some financial subsidies may reduce the cost of insurance under the ACA, many SSDI recipients will still feel the premium is too high, or they may not be getting comprehensive coverage under the ACA plans. If they need specialized medical care related to their chronic condition(s), planning for ongoing care may not be a good option to make under the ACA Marketplace, which would make the plans even less suitable for those who are in need of the ongoing care.

COBRA stands for Consolidated Omnibus Budget Reconciliation Act. It allows people who lose their jobs or move on from other benefits to continue their employer-sponsored health insurance for a limited time. COBRA is an option for SSDI recipients who had employer-sponsored health insurance before being disabled and want to keep that coverage, but the premiums may be unattainable for many SSDI recipients. COBRA premiums can often be considerably higher than what recipients paid when they were working at a job that offered health care. In addition to being expensive, the premiums add stress on individuals when they are already struggling with no healthcare coverage and trying to navigate sudden financial stress and uncertainty. Coverage under COBRA is only temporary up to 18 months and will not cover the entire 24-month waiting period of Medicare to avoid the gap filled from received SSDI benefits for health coverage. Once COBRA coverage expires, they will not have coverage.

Even though programs like Medicaid, CHIP, the ACA Marketplace, and COBRA may provide some level of healthcare access to the individual in the waiting period, these programs do not cover the needs of all individuals who are SSDI beneficiaries. Regardless of the variability of Medicaid coverage, the limited coverage of CHIP, the affordability and options of the ACA to create health insurance, potential barriers of COBRA coverage, and the gaps in coverage that

there still might be, can make it a challenge for many SSDI recipients. Overall these programs seem fragmented, and at times, inadequate. Therefore, it's clear that a more permanent solution is needed to ensure SSDI beneficiaries receive some sort of healthcare services, while they are waiting for Medicare, addressing the bigger picture.

The SSDI Healthcare Access Navigator (HAN) proposal offers a solution to the critical healthcare problems SSDI recipients face when seeking to receive health care during the 24 month waiting period for Medicare. The current options: COBRA, Medicaid and the ACA marketplace options, are inadequate and it is reasonable to suggest that they are also too expensive or too complex to access. While COBRA allows some individuals to keep their insurance after leaving work, the cost is often far too high for SSDI recipients who are already living on limited income. Although the Medicaid option is the most relevant to address the gap, for many it is not available or accessible until their SSDI approval includes coverage for Medicare, creating a gap in medical care coverage for SSDI recipients. Additionally, while ACA marketplace options are available to SSDI recipients, even these can be quite expensive and do not provide the consistent coverage SSDI recipients require due to their disabilities.

The HAN platform addresses these challenges by granting SSDI beneficiaries instant access to affordable or free healthcare options like free clinics and sliding-scale services, helping them navigate the healthcare system to get the care that they need without the long wait for approval or navigating the complex health insurance system. Unlike the current options, HAN provides healthcare without the long wait for insurance, making it more accessible and affordable.

From a financial and healthcare standpoint, HAN may provide long-term cost savings. When SSDI beneficiaries cannot access medical care, their care will get worse, resulting in them making their way to an emergency room and receiving more costly acute care. Identifying these

types of conditions sooner and providing care to these individuals should help to avoid these costly outcomes, thereby positively impacting the health care system. By directing people to healthcare service directly, HAN reduces the burden on Medicaid, which many beneficiaries use as a stop gap during the waiting period.

Assessing the impact of HAN will involve looking at health outcomes, cost savings, and user feedback, as well as the number of clinic visits, which will help to enhance the platform while demonstrating its effectiveness. HAN presents a straightforward and cost-effective way to address the health care needs of SSDI recipients, enabling individuals to get the health care they need while saving costs for the healthcare system. By improving SSDI recipients' access to care, HAN can positively impact their health and overall well-being during a challenging time.

Research questions and target population

The gap in healthcare coverage for individuals approved for Social Security Disability Insurance (SSDI) but still in the two-year Medicare waiting period presents a serious public health and equity issue. Many of these individuals, facing severe medical and financial hardship, struggle to access basic care. In response to this gap, the SSDI Healthcare Access Navigator (HAN) platform is proposed as a tool to improve access to immediate healthcare services. This paper asks: How does the SSDI Healthcare Access Navigator platform impact the immediate healthcare access of SSDI recipients during the Medicare waiting period?

The SSDI Healthcare Access Navigator (HAN) is designed for a targeted audience: people who have been approved for Social Security Disability Insurance (SSDI) and are now in the waiting period for Medicare. Even though SSDI is technically referred to as disability insurance, it's important to note that not just people with visible or physical disabilities can qualify. To be

qualified for SSDI, the individual must have a medical condition that meets Social Security's exacting definition of disability. This definition may include mental, physical, or chronic conditions that have considerable limitations on a person's ability to work. Many of these individuals are not simply dealing with health issues, they are dealing with severe financial stress as well and may not even be able to afford basic healthcare services during the two years. Therefore, they are the main focus of the proposed HAN program that will provide immediate support in accessing free clinics, low-cost services, and other healthcare resources without having to wait for Medicare.

There are also other populations, both positively and negatively affected that could also feel effects besides direct target populations. For example, families and caregivers of SSDI recipients could feel effects positively reducing stress and providing clarity of the pathway to care.

Healthcare providers, specifically those in free or sliding-scale clinics can experience an influx in patients, but they would also be seeing a currently unserved underserved population that is in need. Policymakers and healthcare administrators may also benefit from clearer data and user feedback, which could eventually shape long-term reforms. On the negative side, if the HAN platform is not carefully constructed, there could be negative repercussions for example, the clinics listed on the platform could be overwhelmed if demand suddenly spikes, or recipients might become frustrated if resources listed are outdated or difficult to access. That's why it's critical that the program design is thoughtfully constructed and flexible.

Overall, the program targets SSDI recipients primarily, although it may function as a support for another larger system of care by assisting people with accessing the resources that already exist. The HAN designed platform will eliminate confusion and fragmentation in navigating health care and services that can ultimately reduce barriers, avoid health deterioration, and create some

comfort for a population that is currently stuck in a perilous gap of no care. This proposal aims to show why the HAN program should be implemented and how it could realistically work to improve healthcare access for SSDI recipients during the Medicare waiting period.

Design

One of the main measurable outcomes of the SSDI Healthcare Access Navigator (HAN) program is whether SSDI recipients can find and navigate affordable healthcare during the Medicare waiting period. This is the entire goal of the program, therefore the success of the program should be based upon this measure. The HAN platform is intended to provide beneficiaries with clear, simple, and updated lists of free clinics, low-cost clinics, and mental health resources, thus the simplest and most direct way to measure success is to see whether beneficiaries are using the services they found on the site.

Another important measurable outcome is the percentage of users who report getting care that they otherwise would have skipped or delayed. Many recipients of SSDI have serious chronic conditions and not getting care for months or years can have long-term health and cost implications. If the program can help a portion of their users to not miss medical treatments, or mental health services in some way, then it is achieving an important result. This can be measured using short surveys or intake forms after individuals have received care as a result of information found on HAN.

A third measurable outcome is user engagement with the platform. This engagement includes the number of website visits, the frequency of return visits, the number of clicks on particular clinic links, and the number of newsletter sign ups. These platform engagement indicators represent whether the site is merely a resource that users look at once and forget, or if they are using it as

an active tool or resource. Increasing use over time could also suggest that the physical pamphlets provided as part of outreach are being received, read, and shared with peers that might also benefit from the program. It will be further proof that the program is building trust and relevance in the community if SSDI recipients begin telling the program to other individuals like them.

The program might also measure whether users feel more confident navigating their health in the waiting period. This reflection is more subjective, but it nevertheless matters. SSDI recipients are often in a vulnerable and confusing situation, and simply having someone express, "I know where to go for help" or "I found a doctor I could afford" is an important marker that the program is assisting users in feeling less alone and hopeless.

In total, these outcomes yield both quantitative and qualitative ways to assess whether the HAN platform is accomplishing its purpose: to provide the huge gap that the 24 month period of inactivity imposed by Medicare creates, and assist users to not disconnect from care in the meantime.

For the SSDI Health Access Navigator (HAN) program, the unit of analysis will be individual SSDI recipients who are currently in the two-year waiting period for Medicare. The program's primary focus is to evaluate how the platform assists these individuals in finding access to affordable healthcare, so the sample will be drawn from participants actively accessing the HAN platform. The sample selection will be based on specific inclusion and exclusion criteria in accordance with the objectives of the program.

To recruit participants, the research will focus on SSDI recipients who are currently experiencing the two-year Medicare waiting period since they represent the identified target population for the HAN platform. Only individuals who have visited the HAN platform at least once, and engaged

with healthcare resources available on the site including clicking clinic links, signing-up for a newsletter, or scheduling medical visits are eligible to be included in the analysis. Participants must also agree to voluntarily complete surveys or intake forms used to assess their healthcare access and the effect that the platform has had on their utilization of healthcare.

The study will also exclude people who are not in the Medicare waiting period like those who are already enrolled in Medicare. Participants that have not interacted with the HAN platform or do not want to engage in data collection will also be excluded. Finally, anyone who cannot interact with the platform in a meaningful way because they have privacy concerns or limited access, like people with very limited access to the internet will also be excluded from the sample. Prioritizing people who are engaged with the platform will guarantee that the data collected represents the experience of those who are actually participating in the HAN program, which is critical for assessing the effectiveness of the platform in assisting its recipients in accessing affordable health care during the Medicare waiting period.

As the unit of analysis is at the individual level, it is important to protect the human subjects who take part in the study. The program will follow ethical guidelines for conducting research with human subjects, which include obtaining separate informed consent from each participant prior to the data collection. The informed consent process will explain the study, emphasize that participation is voluntary, any risks involved, and how participants' personal information will be protected. Participants will also be informed that they can withdraw from the study at any time and will not suffer any negative consequences for doing so.

To ensure confidentiality, all survey responses and usage data will be anonymized. Any personally identifying information will be kept separate from the data collected, and findings will be presented in aggregate form to protect individual privacy. In addition, the HAN platform will

employ robust security measures, including encrypted data connections and secure storage protocols, to safeguard participants' personal health information.

For this proposed program, a descriptive research design is the most suitable option. Given the SSDI Healthcare Access Navigator (HAN) platform is in the proposal stage, the primary focus is to propose how to evaluate the program itself, especially how the program might change healthcare access for SSDI recipients in their Medicare waiting period. A descriptive design allows evaluators to identify and describe ways of engagement with the platform, services accessed, and barriers that remain without a comparison or experimental condition.

Considering the expected sample size would be small and drawn from a population with limited access and high need, it seems ethically, and realistically that a descriptive design would be the best fit for this evaluation. The purpose of the evaluation would be to document more details on usage patterns, gather feedback and observe whether users are able to locate healthcare resources that they need via the HAN platform. Users will be most likely connected to the HAN platform through pamphlets, or from someone who was already engaged with HAN. Thus, by following this descriptive lead, we can also determine how many people are using the platform to share their preference for getting health information through their own personal connections and community word-of-mouth.

Descriptive research is effective in the initial stages of the development of a program, where the main focus is determining whether the system is operating as planned and whether it is accessed by the intended target population. In this example, descriptive research would demonstrate whether or not SSDI recipients use the linkages, what types of service linkages they are able to access, and whether access to the services lessens the negative impact of the 24-month Medicare waiting period. The later evaluations might be best equipped to conduct causal analyses, or look

at certain interventions. At this stage, descriptive research captures the context that enables feedback on users' actual experiences.

The descriptive design for the SSDI Healthcare Access Navigator (HAN) program is ideal for answering the research question "If implemented, how might the SSDI Healthcare Access Navigator (HAN) impact the immediate healthcare access of SSDI beneficiaries during the Medicare waiting period?". Because it will capture the use of the HAN as implemented, the descriptive design will allow for detailed documentation of user engagement, which is essential for understanding the ways in which recipients engage with the platform, whether it provides intended access to healthcare.

With this design we will get real time data on which healthcare services SSDI beneficiaries can access, how often and the barriers or issues they run into seeking affordable care in the Medicare waiting period. Users will also be able to provide feedback regarding their experience using the site and how the platform resources like free or low-cost clinics are addressing their healthcare needs. The study design is modeled after the program and relies on referrals via pamphlets, recommendations from other participants currently using the HAN connecting platform, and so it will follow the natural flow of the program into communities. This will give a comprehensive view of how the HAN is impacting users from their perspective, asking how the HAN is affecting healthcare access directly. In addition, collecting data on use patterns as well as user feedback will help answer the question of whether this program is successfully simplifying healthcare access in a significant waiting period time.

The SSDI Healthcare Access Navigator (HAN) program shows how it is intended to work in Exhibit 1-E. The basic concept is that the HAN website connects SSDI recipients to free and low-cost clinics while they await Medicare. The program is straightforward in its goal of

connecting SSDI recipients to affordable healthcare right away. The model also highlights how the HAN program has larger implications, by allowing more people to access care sooner, more people will be connected to the care they need and fewer individuals will go without treatment during the waiting period. This ultimately means SSDI recipients will have better health and more stability. The HAN platform is designed to make a difference by linking people to their healthcare options.

Data: Operationalization and Analysis

To make the SSDI Healthcare Access Navigator (HAN) program work, it's important to identify both the inputs and outputs at proximal and distal levels. Inputs are the resources and activities needed to implement the HAN Program. Inputs include not only the creation, operation, and maintenance of the HAN platform, but the partnerships established with free or low to no cost clinics, and the distribution of information like pamphlets and newsletters, while also including the oversight, coordination, and management of staff, clinic data input into the platform, and upkeep of resources, and information pertaining to healthcare resources that is accurate and accessible to SSDI recipients.

The proximal outputs are the intended immediate outcomes of the program implementation. The proximal outputs aim to evaluate the immediate effectiveness of the HAN platform in linking SSDI recipients to health resources. Examples include the frequency of website usage, the number of recipients accessing healthcare services through the platform, and the level of user satisfaction with the resources provided. The proximal outputs will help assess whether the platform enables immediate access to health care for those awaiting Medicare.

The distal outputs are the longer-term goals that the HAN program hopes to achieve. These outcomes will be evaluated over a longer timeframe and will consider the more general health

and well-being characteristics of SSDI recipients. The distal outputs include improvements in recipients' overall health due to timely medical care, fewer emergency health care visits, and greater financial stability and quality of life. These distal outputs will provide insight into the program's success in addressing the long-term healthcare needs of SSDI recipients during the waiting period.

The evaluation of the SSDI Healthcare Access Navigator (HAN) program will rely on both primary and secondary data to develop a more comprehensive understanding of the effectiveness of the platform to access healthcare services for SSDI recipients during the waiting period for receiving Medicare benefits.

Primary data is information obtained directly from participants or users of the HAN platform. In the case of this program, primary data will be collected and utilized through surveys, interviews, and user feedback forms. These data gatherers will provide insights into the experience of SSDI recipients of the HAN platform, including information on how they access health services, if they are satisfied with the resources available to them, and whether they actually got the care they needed while waiting. Surveys will be sent out to users of the HAN platform to collect data on the health care needs of users, obstacles users faced, and if their access to care improved overall after using the platform. This first-hand data is essential for us to clearly understand the outcomes of the program, including level of user involvement and user satisfaction.

Secondary data consist of information that another researcher or organization has previously collected and evaluated; for this program, potential sources of secondary data can include existing evaluations of similar programs, studies on healthcare access for under-served populations, or evaluations of other digital health navigation tools. The secondary data will play an essential role by giving context to the HAN program and comparable data to evaluate the

effectiveness of the HAN program against other initiatives. Existing government reports such as SSDI utilization rates or healthcare utilization among low-income populations will also be a source of secondary data. Each of the sources of secondary data will provide context for understanding the impact of the HAN platform as it relates to the overall area of healthcare access and disability support services.

Utilizing both primary and secondary data will be important to enable a balanced assessment of the HAN program. Primary data gathered will provide direct knowledge based on how users experience the HAN platform, producing learnings on its immediate impact through users' representation. Also, this data will provide insights into the experiences and implications for any SSDI recipients using services through HAN, and potentially their healthcare needs while waiting for services through the SSDI process. Secondary data will offer a perspective from global, national or state similarities in other similar or same programs, including comparisons of trends relating to access to healthcare for SSDI recipients. Together, all data will provide concluding knowledge on the extent to which the program is successful and the changes required to improve it.

Along with the evaluation design and data sources identified, we needed to think about how valid and reliable those measures will be in capturing the actual impacts of the program. There are a few important reliability and validity considerations of the selected measures that we need to pay attention to when evaluating the SSDI Healthcare Access Navigator (HAN) program.

Because evaluation relies on self-reported data by users like surveys and feedback there is the chance of variance depending on how the questions are framed or how participants may have interpreted the questions. For example, people reporting their health care experiences may under provide or overprovide because of their mood, memory, or expectations about the program. The

program will also likely draw from a small and diverse sample group with limited numbers of participants and perhaps little consistency across groups or times of measurement. Some users may have filled out forms with some inconsistencies because of literacy issues or technological barriers leading to inconsistent reporting of the data.

There is also an issue of whether the measure intended to tap into the outcome really identifies the intended outcome. For instance, asking if someone “found healthcare” is unlikely to tap into the quality or timing of that care. Simply finding a clinic or doctor’s office does not mean they were treated or had their health goals improve. Additionally, because many SSDI recipients experience a myriad of health needs, financial barriers, and poor health, it could be a struggle to independently attribute any change in these individuals' circumstances as a result of HAN, and not as other unrelated contributing factors. As such, isolating the actual changes brought about through the program becomes increasingly more difficult to identify and is further complicated by the potential introduction of bias.

The proposed evaluation design provides relevant information about user experience and access to healthcare, but care must be taken with what may be claimed with certainty from the findings. Transparency about these limitations will help ensure the findings are not over interpreted and the next evaluations will have the opportunity to improve on the tools being used.

While these issues regarding reliability and validity are important, careful planning of how data are analyzed contingently can be extremely useful. Applying clear and appropriate analytical techniques can illustrate patterns of access and access in the evaluation which may represent evidence of a program working as intended.

The evaluation of the information gathered from HAN users will mainly look at the data collected using descriptive statistical methods. For example, calculated frequencies and

percentages will be used to show how many users accessed resources, which service types were accessed the most, and how frequently users reported improvements in their ability to obtain care.

Besides numerical summaries, qualitative feedback from open-ended survey questions will be analyzed through content analysis. This allows for repeated themes in user experiences and recommendations to improve the platform to be identified. By putting both types of data together the analysis creates a comprehensive picture of how the SSDI Healthcare Access Navigator is working, and where it may need to be adjusted.

Conclusion

If the HAN platform is implemented, it will effectively address a major access to care gap in health care for SSDI participants who are stuck in the 24-month waiting period for Medicare. Due to a lack of insurance, many SSDI participants are subject to financial and health-related hardship during this timeframe. By providing users with a single platform centrally locating free and low-priced clinics, the HAN program could effectively help alleviate the immediate stress created by this coverage gap and provide encouragement in finding the resources available to them.

The program's design is grounded in the belief that a clear, centralized, and easily navigable website can meaningfully improve the ability of SSDI recipients to access healthcare. If the HAN platform works as intended, it will help users find services based on location and cost, allow users the option to subscribe for updates to service, and allow the user to have the least bureaucratic barrier possible that most programs require. Pamphlets, along with conversations

with existing consumers, will be used to promote its adoption. If each of these works, then the program should achieve its intended and intended responsiveness.

In terms of process, if the HAN website is designed to be accessible and user-friendly, this may allow a seamless experience for those using the platform who have lower literacy or unreliable internet connection. The program's success will also rely on recipients actually learning about the platform, whether it be through flyers in public clinics or from other SSDI recipients that have navigated the site. If these informal networks succeed in increasing awareness and use, the HAN platform could potentially be part of the ordinary navigation experience of SSDI.

If the HAN program were to be implemented and distributed widely, its biggest opportunity to make an impact would be that more SSDI recipients could receive timely medical care. The goal of the program is to improve access to health care and, therefore, limit the number of individuals who do not receive productive medical treatment, or who see their health deteriorate while waiting for Medicare. Any improvement in access could mean more opportunities for welfare, diminishing reliance on emergency services, and a greater degree of stability for recipients and their family members.

Finally, if the HAN platform is inexpensive and easy to maintain, especially in relation to the public health and social benefits, it could be viewed as a low-cost model. HAN avoids the cost of creating new services and approaches value to high need populations by leveraging existing healthcare assets and only adding visibility and connectivity. The ultimate test of its efficiency would be whether the costs of operating the HAN were offset by a reduction in unserved conditions and emergency healthcare costs.

In evaluating the possible impacts of the HAN platform, some design and evaluation caveats should be discussed as they may limit the overall utility and success of the platform. Limited

access to the internet is a critical caveat, as it is likely that not all SSDI beneficiaries have sufficient internet access or competence in using online tools, could limit the platform's reach. Language and access challenges may include exclusion from specific groups or communities, especially if the platform does not fully leverage these barriers, leaving some individuals unable to engage with the platform. The usual caveat of a survey being limited may need to consider that not every user will complete a survey or may provide a biased response based on their specific history with or experiences of the platform. As such, these caveats raise important concerns about the potential accessibility and feedback methods of the HAN platform.

One of the key strengths of the SSDI Healthcare Access Navigator (HAN) proposal is its emphasis on a critical gap in healthcare access for SSDI recipients during the Medicare waiting period. Existing literature discusses various components of SSDI recipients' challenges, but little research exists discussing practical, immediate solutions for accessing healthcare during this waiting period. HAN fills this gap by providing a centralized service that connects SSDI recipients to free or low-cost healthcare services before they wait for Medicare approval. Additionally, the program's emphasis on real-time feedback and user-driven improvements helps ensure that the platform evolves to meet the diverse needs of its users, further enhancing its relevance and effectiveness. Because of this, the user-centred development approach is a strength in that it tailored the program to the lived experiences of users and not enough is known about real-time user-centred development in evaluations of existing SSDI healthcare access initiatives. The SSDI Healthcare Access Navigator (HAN) program has many significant implications for policymaking and future research. From a policy perspective, if the HAN was successful, it could demonstrate the value in allowing SSDI recipients to immediately access healthcare resources during the Medicare waiting period, and may serve as a catalyst for policymaking to

better engage this population. In addition, if the HAN program yields positive results in access to healthcare and in health outcomes, it may not only serve as a potential model for furthering similar programs, but could also inform expansive policy discussions intended for improving access to healthcare for vulnerable groups, when online platforms, such as the HAN are utilized. This may include proactive policies addressing gaps in the healthcare system for those waiting for long-term coverage, such as SSDI recipients.

Studying HAN offers an opportunity to build knowledge on healthcare access for underserved people. By investigating how effectively digital tools and resource hubs work, the program will provide important information about improving health outcomes for people who experience limited access to traditional healthcare avenues. In addition, the findings could support other studies into how to design and implement online tools for other vulnerable groups subject to similar barriers. The findings from HAN could offer lessons for future studies at the intersection of digital health services and social security programs, and expand the broader understanding of equitable access to healthcare.

Communication plan

The evaluation feedback of the SSDI Healthcare Access Navigator (HAN) program will be disseminated through primary and secondary methods to maximize visibility and engagement from participants, healthcare providers, and researchers.

The primary distribution plan will be to report findings back to the program's target audiences and specific stakeholders. This will be accomplished through workshops or webinars, which will be customized specifically to SSDI recipients, healthcare providers, and advocacy organizations. These sessions will provide an opportunity to present findings, receive feedback, and discuss

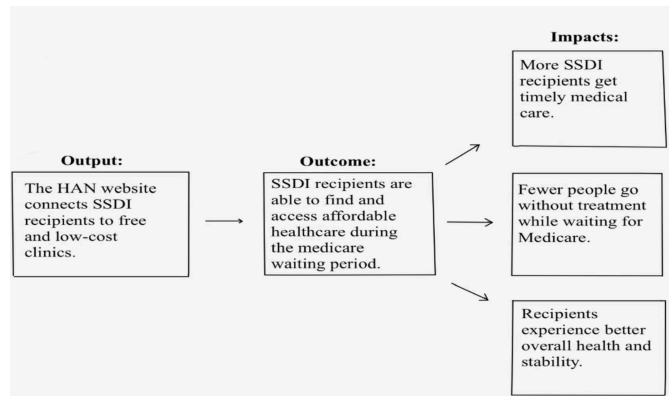
recommendations. Once the findings have been presented, detailed reports and executive summaries will be sent to the various stakeholders, SSDI recipients, healthcare practitioners, advocacy organizations, and social security program administrators. The reports will summarize the key findings, and provide highlights of the recommendations available for action. The surveys will be distributed to participants to gather feedback on their experiences with HAN. Findings from the surveys will be communicated directly to users, stakeholders, and service providers to take back for continuous improvement.

Secondary dissemination will consist of the sharing of the evaluation outcomes with a wider audience who will be able to engage with the outcomes to promote ongoing research and policy discussions. As sharing the results with a wider audience will be valuable to build on the existing evidence base, this will consist of submitting results for publication in scholarly journals that focus on issues of healthcare access, or social security, or digital health services. This will allow for more thorough analysis and present additional evidence in the literature. This will involve presenting the results at conferences or events where the audience is primarily composed of researchers and policymakers focused on healthcare policy, specific social security programs, or the broader digital health space. This will allow participants to engage with the results and further contextualize those results and potential implications for their work. Finally, the program's findings will be shared in the form of a program poster. This will be presented at academic, healthcare, or advocacy conferences relevant to the context of the presentation. The poster will provide a visual delineation of the inquiry's key outcomes and impact, making the findings available to stakeholders from healthcare professionals to policymakers.

While most stakeholders are likely to validate the SSDI Healthcare Access Navigator (HAN) program proposal and subsequent evaluation, as outlined in the background and program model,

some stakeholders may have reservations related to accessibility and implementation challenges. SSDI recipients will be valuing the program, offering direct connection to a low-cost option for accessing affordable healthcare, however, barriers such as lack of access to high-speed internet will affect their level of adoption. Healthcare providers will likely support parts of the program, but they'll also want to know whether it will bring in more patients and how that might impact their capacity. Advocacy groups are likely to support the program because it aligns with their mission to help underserved communities. Policymakers may adopt a more limited view of the proposal and will be concerned with issues of scalability and responsible spending. Researchers will see this evaluation as a good opportunity to explore issues around healthcare access for vulnerable populations for future studies. Collectively, stakeholders will appreciate the efforts and longstanding commitment to supporting underserved populations but recognizing accessibility may be a barrier to their mission as funders and implementers.

Exhibit 1-E:



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